

Clinical features

Specific manifestations include intense itch and disseminated inflammatory papules. Non-specific manifestations which may also occur are skin excoriation, secondary eczematization, and impetiginization.

1. Classical scabies

- Occurs in patients with normal immune response
- Intense pruritus, worse at night
- Erythematous papules disseminated on the periumbilical area, waist, genitalia, breasts, buttocks, axillary folds, fingers (including interdigital spaces), wrists, and extensor aspects of the limbs. The head, palms, and soles are usually spared in adults
- Papules are small, often excoriated, with haemorrhagic crusts on top
- The burrow (a pathognomonic sign) appears as a thin, brown-grey line of 0.5–1 cm, but is rarely observed due to excoriation or secondary bacterial infection
- Other lesions: vesicles (usually at the start of a burrow), nodules (firm, 0.5 cm in diameter, usually on the male genitalia, groin, buttocks), weals
- Poor hygienic conditions may result in secondary bacterial infection
- Irritant or allergic contact eczema can be induced following topical treatment

2. Crusted scabies (the term 'Norwegian scabies' should no longer be used)

- Occurs in patients with severe immune deficiency due to disease (e.g. AIDS, HTLV-1 infection, malignancy, leprosy) or therapy (e.g. immunosuppressant drugs and biologicals), neurological disease causing reduced sensation,

immobility with reduced ability to scratch, or in genetically susceptible patients

- Pruritus is mild or absent
- Skin lesions consist of generalized, poorly defined, erythematous, fissured plaques covered by scales and crusts. On bony prominences (e.g. finger articulations, elbows, iliac crest), the plaques have a yellow-to-brown, thick, verrucous aspect
- Diffuse non-crusted scabies with involvement of the back may also occur
- Bacterial secondary infection can result in malodorous skin lesions

Diagnosis

Diagnosis is suspected based on the characteristics of itch (generalized, intense at night), clinical findings, and a suggestive history (e.g. exposure context, disease in close contacts). Definitive diagnosis is supported by a positive microscopic examination of skin scrapings, which may identify mites, eggs, or faecal pellets (scybala). To enhance detection, the burrow can be opened with a fine needle and Müller's oil or immersion oil applied to bring the mite to the surface. A negative microscopic result does not exclude scabies.

Dermoscopic examination can identify skin burrows and mites (the 'delta' sign at the end of the burrow represents the anterior body of the adult female mite), as well as eggs, and can help guide the site of skin scrapings. In sexually active patients, STI screening (including HIV test) is recommended.

General principles of treatment

Ten new clinical trials on scabies treatment have been published since the previous guideline in 2010. Recent data focus on mass population treatment, usually with ivermectin. Based on existing comparative studies on the efficacy of different antiscabetic treatments, a distinction is made only between

'recommended' and 'alternative' treatments. The availability of antiscabetic drugs varies across European countries, leading to variability in treatment choices in practice.

Topical treatment should be applied to all skin areas, including the scalp, groin, navel, external genitalia, finger and toe web spaces, and beneath the nails. It should be applied at night and left in place for 8–12 hours. The skin should be cool and dry before application. A second application is recommended after 7–14 days.